



Fiona Stanley Fremantle Hospitals Group

Policy and Procedure

Patient Identification

Reference #: FSFH-HW-POL-0034

Scope

Site	Service/Department/Unit	Disciplines
Fiona Stanley Hospital Fremantle Hospital	Hospital Wide	All Disciplines

1. Introduction

All patients accessing services or being admitted to Fiona Stanley Hospital (FSH) and Fremantle Hospital (FH) will be identified using the procedures set out below.

The policy and procedure outlines the steps required to confirm patient identity on arrival and during the episode of care.

2. Terminology

Clinical photography	Clinical photography illustrates a patient's condition or an aspect of the treatment, to form a part of the patient's medical record.
Medical photographic system (MPS)	Purpose built database to allow Health Care professionals access to medical photographs at workstations within Fiona Stanley Hospital.
Photographic identification	Use of a photographic image of an individual patient in order to accurately identify the patient when they are unable or unwilling to wear an approved identification band.

3. Policy and Procedure

3.1. Patient identification descriptors

- All patients admitted to FSFHG will be identified using a minimum of three core patient identifiers:

- Family name and given name/s. which is their full legal name
- Date of birth.
- Unit Medical Record Number (UMRN). Address may be used if UMRN is unknown.
- webPAS generated patient identification labels must be produced for each patient as a record of patient identifiers.
 - Labels will be used by staff on all documentation and laboratory specimens.
 - Family and given names must be clearly differentiated. The family name must appear first using upper case letters followed by given names in lower case (e.g., SMITH, John Paul).
 - The Admissions/Ward Clerk or equivalent must ensure appropriate supplies of patient identification labels are always available for use in the medical record.

3.2. Confirmation of patient identity

- Positive identification of all patients (including Emergency Department patients, Day Procedure patients, and Outpatients) is undertaken by relevant staff:
 - At the time of admission/registration.
 - Prior to the provision of any care, therapy or services, including but not limited to:
 - all procedures
 - sample collection
 - investigations
 - all medication administration
 - meal delivery
 - administration of Expressed Breast Milk (EBM)
 - Whenever clinical handover, patient transfer or discharge documentation is generated.
- Patients are asked to state their family and given names (requesting they spell these out where necessary), date of birth and address.
- If the patient is unable to give this information, it must be recorded in the medical record and all reasonable attempts must be made to confirm the patient's identity which can include:
 - confirming with an accompanying adult
 - checking alternative identification (e.g. driver's licence)

- use of an interpreter.
- Neonates transferring from other sites are to have their identity confirmed with the accompanying health professional.
- Be aware of cultural naming conventions and the use of preferred names rather than legal names when determining identification information.
- Aliases are to be added to webPAS for patients who prefer to use alternative names. (Aliases for Mental health patients are to be added to PSOLIS also).
- Interpreting services can be accessed to assist in confirming the identification of consumers (and their carers) who speak limited or no English, who are deaf or hard of hearing following the “FSFHG Interpreter Booking” Policy and Procedure.

3.3. Unidentified patients

- Where a patient’s identity cannot be reliably confirmed, they must be registered as ‘Unknown Male’ or ‘Unknown Female’ using an emergency UMRN generated by webPAS.
- Once the patient has been identified, the patient’s information must be updated in webPAS, a new identification band must be attached to the patient, new patient identification labels are generated, and the old labels discarded in confidential waste.
 - Where required the correct patient must be identified and merged with the unknown patient in webPAS and the Digital Medical Record (DMR).
 - The ward clerk must notify, as a matter of urgency, all departments that have performed investigations and treatment (i.e. imaging, pathology, pharmacy) to update the patient details.

3.4. Patients with same or similar names

- Particular care must be taken when there are two or more patients in one ward or department with the same or similar names.
- Staff members are to ensure that the appropriate signs and labels are always in place whilst both patients remain in the ward.
- **“Caution – Patient with same or similar name”** magnets must be displayed on Bed Name Cards (above the head of their bed), Patient Bedside Board or any other board that shows the patient / patient/s name/s.
 - On Journey Board this is indicated by ‘Patient with same or similar name’ icon 
- **“Caution – Patient with same or similar name”** stickers must also be attached to the cover of the patient’s medical folder, and on all medication charts and

other forms where medications or treatments are prescribed (i.e., anticoagulation chart fluid treatment chart, etc.).

3.5. Identification bands

3.5.1. General Information and Procedure

- A single white or red (allergy) ID band must be used for patient identification (unless meet the criteria in 2nd paragraph) and is to remain on the patient throughout their hospital admission.
- **Two ID bands are required for Neonates (see 3.7 Neonates), patients in the Intensive Care Unit (ICU), patients attending the operating theatre and patients undergoing a procedure under sedation or General Anaesthetic.**
- Red is used to denote any type of allergy (e.g., medication, dietary, latex).
 - The red patient ID band must not contain details of the allergy. This information must be recorded on the patient's medical records.
 - For Mental Health Areas and Hyperbaric unit, see 3.6 Situations where ID bands are not worn and 3.8 Photographic ID.
- The ID band is to be prepared by the admissions/ward clerk or equivalent.
- All inpatients, day procedure patients and Emergency Department patients will have an ID band securely attached immediately after patient registration/admission and before any:
 - treatment
 - procedure
 - collection of pathology samples
 - blood transfusion
 - drug administration, or
 - imaging procedure
- Nursing / midwifery staff check the ID band (making sure if there are any allergies that the band is red) and ensure it has been placed on the patient's wrist or ankle as appropriate, as soon as practicable after the patient arrives in the Emergency Department, on the ward or treatment area.
- The patient must view and verify that the details of the ID band are correct. If the patient is unable to do this, their next of kin / legal guardian / carer may do so on the patient's behalf; otherwise, a second staff member is to check the ID band against the patient's DMR or medical folder.

3.5.2. Removal of the second ID band must take place following:

- Transfer back to the ward from the Operating Theatre or procedure with sedation/general anaesthetic, which, is to be documented in the PACU Nursing Handover chart.
- Upon the receipt ward following discharge from ICU.

3.6. Situations where no ID bands are worn

- There may be situations where it is not practicable for a patient to wear a patient ID band including but not limited to:
 - A patient who is cared for in a mental health setting, see 3.8 Photographic ID;
 - A patient with acute or chronic confusion/agitation; and,
 - A patient that cannot wear an ID band due to a clinical condition or treatment.
- In the situations stipulated above confirmation of the patient's identification may include any of the following:
 - Verbal confirmation of full name, address and date of birth checked against the referral or any medical record or URMN.
 - Photo identification (See 3.8 Photographic ID)
 - Verbal verification by carer, family member, friend, ambulance, police where a patient is unable to identify themselves, where there may be uncertainties about identification, or the patient is a minor/neonate.
 - Identification confirmation performed by two staff that can reliably identify the patient. Details of these staff member(s) are to be recorded in the patient's record.

3.7. Neonates

- Neonates must wear two white ID bands at all times.
- Both neonatal patient ID bands are placed on the neonate at birth, prior to leaving the birthing area (Birth Suite/Theatres / Family Birth Centre (FBC)) to indicate the newborn's unique UMRN, Date of Birth (DOB) and including mother's details in brackets.
- Staff are to confirm with the mother and/or father that the details of the neonate's ID bands are correct prior to placing on the baby.
- If the sticker with the baby's UMRN is unavailable, then a mother's sticker with "B/O" clearly written and date of birth of baby indicated should be used and then replaced as soon as the baby's sticker is available.

The neonate is not to leave any area without having 2 ID bands in situ.

- On admission to the ward the neonate's ID bands must be checked by the receiving Midwife/Nurse against the mother's ID band (where in attendance) with the transferring Midwife/Nurse.
- If the neonate is removed from the mother's bedside by staff, the ID bands of the mother and neonate must be re-checked and identified as correct when the neonate is returned.

The neonate's ID band must not be changed during the first admission, even if the birth has been registered with the State Registry of Births, Deaths and Marriages: If this original ID band is changed Transfusion Medicine will not issue any blood products, should they be required, due to the original ID from birth changing.

- Expressed breastmilk bottle labels should be checked against the infant's ID band by two Nurses, or Nurse and infant's carer, prior to administration of breast milk to the infant and during the logging out of breast milk from the Level 3 Nutrition Room fridge or freezer.

3.8. Photographic ID

- Patients must consent to their picture being taken, and consent practices must comply with the [WA Health Consent to Treatment](#) policy, the FSFHG Correct patient identification, consent and procedure matching policy and FH Photographic patient identification.
- For photographic patient ID in Mental Health at FH please refer to the FH Photographic patient identification policy
- All patient identification photography is to be conducted by the Medical Illustrations Department (where possible) and requires an FSFHG photography authorisation request form. The photography authorisation request form is to be completed prior to portrait being captured and is to be submitted to Medical Records for scanning.
 - no portrait /clinical photography is to be taken without a completed form

3.9. Boarders

3.9.1. Babies admitted as boarders

- All babies admitted as boarders must wear two identification bands at all times.
- The ID bands are to be checked with the parent/guardian on admission.

3.9.2. Adult Boarders

- All adults admitted as boarders must wear an identification band at all times to enable checks of ID against them. More information about boarders is available [SMHS Boarders in Hospital Policy](#).

3.10. Patients in Operating Theatre/Day Procedure

- ID bands should be placed on the wrist and on the ankle of the opposing side to the wrist band where possible.
- ID bands are not to be placed on the side of the proposed procedure if the patient is undergoing upper or lower limb surgery.
- If the patient is undergoing bilateral limb surgery, the ID bands should be placed on non-operative limbs.
- Patient identification is an important part of the Team Time Out and handover processes in Theatre and procedural areas. Identification is confirmed in Team Time Out processes including the use of Surgical Safety Checklist as per “FSFHG Correct patient identification, consent and procedure matching policy and procedure”. (FSFH-OPET-POL-0008)

3.11. Removal of ID band due to medical procedures

- If ID bands interfere with medical procedures and are removed, they are to be reattached as soon as practicable following confirmation of identification against the patient’s DMR by the person removing the original band.

3.12. Hyperbaric Unit

- Initial confirmation of ID of patient is conducted as per 3.2 Confirmation of Patient Identity.
- The hyperbaric unit uses photo ID which is arranged as per the “Quick reference guide for obtaining a clinical ID portrait for the Hyperbaric Unit”.
- A patient ID label is placed on the back of the photo ID to enable continued three-point ID checks prior to procedure, or administration of medication.
- This Photo ID is used for the duration of the patient’s treatment plan and then disposed of in the confidential shredded waste bin at conclusion of treatment.

3.13. Procedural areas

- Inpatients
 - Identification prior to procedure is to be conducted checking three patient identifiers against the ID band and verbally with the patient.
- Outpatients

- Initial confirmation of ID undertaken by clerk using three identifiers – name, DOB and address if no UMRN is available.
- Procedure lists continue the ID process verbally with the patient using three patient identifiers, prior to undertaking procedures.

3.14. Porter/Patient Service Assistance (PSA) patient ID process

- Requests to transport a patient without a clinical escort require details including patient name, UMRN and DOB. These fields are captured upon request either through calling the Helpdesk or logging a self-service job.
- When a Porter/PSA receives such a request, the Porter/PSA:
 - attends the ward or department and consults with the ward clerk or clinical staff before entering the patient room
 - confirms the identity of the patient by asking the patient to state their (1) first name and last name, (2) DOB and (3) sights their UMRN. If the patient is unable to provide this information, the porter will receive this information from a clinical team member.
 - If there is no job logged but the Porter has been asked to move a patient and therefore does not have 3 points of ID to reconcile against, the Porter will ask a Clinician to confirm the patient ID prior to the move occurring.
 - If there is a clinical escort assigned to a patient transport, the Porter/PSA is not required to perform the three-point patient identification, as the responsibility lies with the Clinical Escort.

3.15. Deceased patients

- See FSFHG Care of the Deceased Adult policy and procedure. (FSFH-HW-POL-0087)

3.16. Incorrect identification of a patient

- On recognition that the patient is incorrectly identified, the ID band issued with incorrect patient details must be removed and a new ID band with the correct details must be issued and attached to the patient immediately.
- The replacement ID band must be checked with the patient.
- The patient's DMR and all associated documentation must be corrected in accordance with the FSFHG Clinical Documentation Policy.
- The event must be thoroughly documented in the patient's DMR.
- A Datix Clinical Incident Management System (CIMS) entry must be completed.

- All departments that have performed investigations and treatment, such as Imaging, Pathology and Pharmacy, must be notified as a matter of urgency.
- The most senior member of the clinical team on duty must ensure the patient involved in the incident is safe, open disclosure has occurred and that all necessary steps have been taken to support and treat the patient and to prevent injury to others.

3.17. Clinical Psychology and Neuropsychology Patients

- All inpatients and outpatients seen by a Clinical Psychologist or Neuropsychologist for an initial assessment session require a 3-point ID check to be completed & documented.
- For inpatients & outpatients continuing psychological treatment with the same Clinical Psychologist or Neuropsychologist who completed their initial assessment – 3-point ID check to be documented as “Patient engaged in ongoing psychological treatment, and they are well known to author”.
- For patients who transition to a new Clinical Psychologist or Neuropsychologist at any time during their psychological assessment or psychological treatment, - new clinician must complete and document a 3-point ID check.
- Routine 3-point ID checks to occur with telephone only clinics with a Clinical Psychologist or Neuropsychologist.

4. Compliance/performance monitoring

- The FSFHG Communicable for Safety Committee will be responsible for monitoring compliance with this document via monitoring of identification processes and monthly reporting and monthly reporting to the Clinical Governance Committee.

5. Related policy documents

- WA Health:
 - [State-wide-Standardised-Clinical-Documentation for \(SSCD\) for Mental Health Services](#)
- SMHS:
 - [SMHS Borders in Hospital Policy SMHS: 25](#)
 - [Clinical photography and videography SMHS: 81](#)
- FSFHG:

- Care of the Deceased adult (FSFH-HW-POL-0087)
- Clinical Handover (FSFH-HW-POL-0035)
- Clinical Documentation (FSFH-HW-POL-0039)
- Correct patient identification, consent and procedure matching (FSFH-OPET-POL-0008)
- FSFHG Blood transfusion policy manual (FSFH-HW-POL-00460)
- Interpreter Booking (FSFH-HW-POL-0006)
- Medication Administration (FSFH-HW-POL-0022)
- Neonatal medication administration (FSFH-NEO-GUI-0010)
- Neonatal Death (FSH-NEO-GUI-0003)
- Patient identification for delivery of meals, drinks and snacks (FSH-ALLH-POL-0002)
- Photographic patient identification (FSFH-MENH-POL-0028)

6. Related standards

NSQHS

- Communicating for Safety
- Comprehensive care

7. Authorisation

EXECUTIVE SPONSOR: Nurse Director Service 2					
Version	Date Issued	Compiled/Revised By	Committee/Consumer Group Consulted	Endorsed By	Revision due
1	02/2016	Nurse Director	Hospital Executive Committee	FSH Policy Committee	02/2019
2	11/2016	Nurse Director	Hospital Executive Committee	FSH Policy Committee	02/2019
3	10/2018	Nurse Co-ordinator	Patient Identification and Procedure Matching Committee & NMPGC	FSFHG Policy Committee	10/2021
4	12/2020	Nurse Unit Manager, Neonates	Nursing and Midwifery Practice Governance Committee; Communicating for Safety Committee	FSFHG Policy Committee	10/2021
5	04/2022	Health Standard Lead, Communicating for Safety	Communicating for Safety Committee, NMPGC	FSFHG Policy Committee	04/2024

6	04/2024	Health Standard Lead, Communicating for Safety	Communicating for Safety Committee,	FSFHG Policy Committee	06/2026
6.1	05/2024	Health Standard Lead, Communicating for Safety	Minor Update, No Change to Review Date	FSFHG Policy Committee	06/2026
6.2	06/2024	Health Standard Lead, Communicating for Safety	Minor Update, No Change to Review Date	FSFHG Policy Committee	06/2026
6.3	08/2024	Health Standard Lead, Communicating for Safety	Communicating for Safety Committee Minor Update, No Change to Review Date	FSFHG Policy Committee	06/2026

8. Appendices

8.1. Appendix 1: Photographic ID after-hours

For an **urgent** identification portrait, the Clinician is to ensure the correct photograph is matched for patient identification purposes and:

- Obtain camera kit from After-Hours Nurse Managers Office.
- Photograph the patient's unit medical record number (UMRN) sticker for identification purposes. This information is required to be entered into the Medical Illustrations MPS database for record keeping. The processing of the identification portrait will not proceed if this information is not provided.
- Take the patient's portrait photograph.
- Connect the camera to PC to upload the image via MyFile Transfer.
- Return the reference book and camera to the kit and return the camera kit to the After-Hours Nurse Managers Office.

For **non-urgent** identification portrait the Clinician is to ensure the correct photograph is matched for patient identification purposes and:

- Obtain camera kit from After-Hours Nurse Managers Office.
- Document the following information in the reference book before taking a photograph of the patient's unit medical record number (UMRN):
 - Patient identification sticker containing the three key identifiers, name of patient, UMRN and date of birth
 - Diagnosis – document 'ID photograph' in this field
 - Requestor name
 - Department/ward
 - Date image was taken
 - Quantity of images taken
 - photograph the patient's UMRN
- Take the patient's portrait photograph.
- return the reference book and camera to the kit and return the camera kit to the After-Hours Nurse Managers Office.

8.2. Appendix 2: Processing the photographic ID

Urgent identification portraits:

- The clinician will notify Medical Illustrations of a request for an urgent identification via email to fsh.medicalillustrations@health.wa.gov.au
- Medical Illustrations will receive the MyFile Transfer email containing the image and patient details. The requester will be notified once the image has been downloaded.
- The photographer will obtain the patient image and patient details to process in accordance the SMHS Clinical Photography and Videography Policy
- The identification portrait will be available to view on MPS via deep link to the patient digital medical record and forwarded to the relevant ward/department to be placed in the medical record

Non-urgent identification portraits:

- The Medical Photographer will collect the FHMHS camera every Monday and Thursday to process the new patient images and place them into MPS for viewing
- The photographer will obtain the patient image and patient details to process in accordance the SMHS Policy Clinical photography and videography.
- This information is entered into the Medical Illustration database for record keeping and is connected to the patients' medical record. Processing will not proceed if this information is not provided.
- The identification portrait will be available to view on MPS via deep link to the patient digital medical record and forwarded to the relevant ward/department to be placed in the medical record.